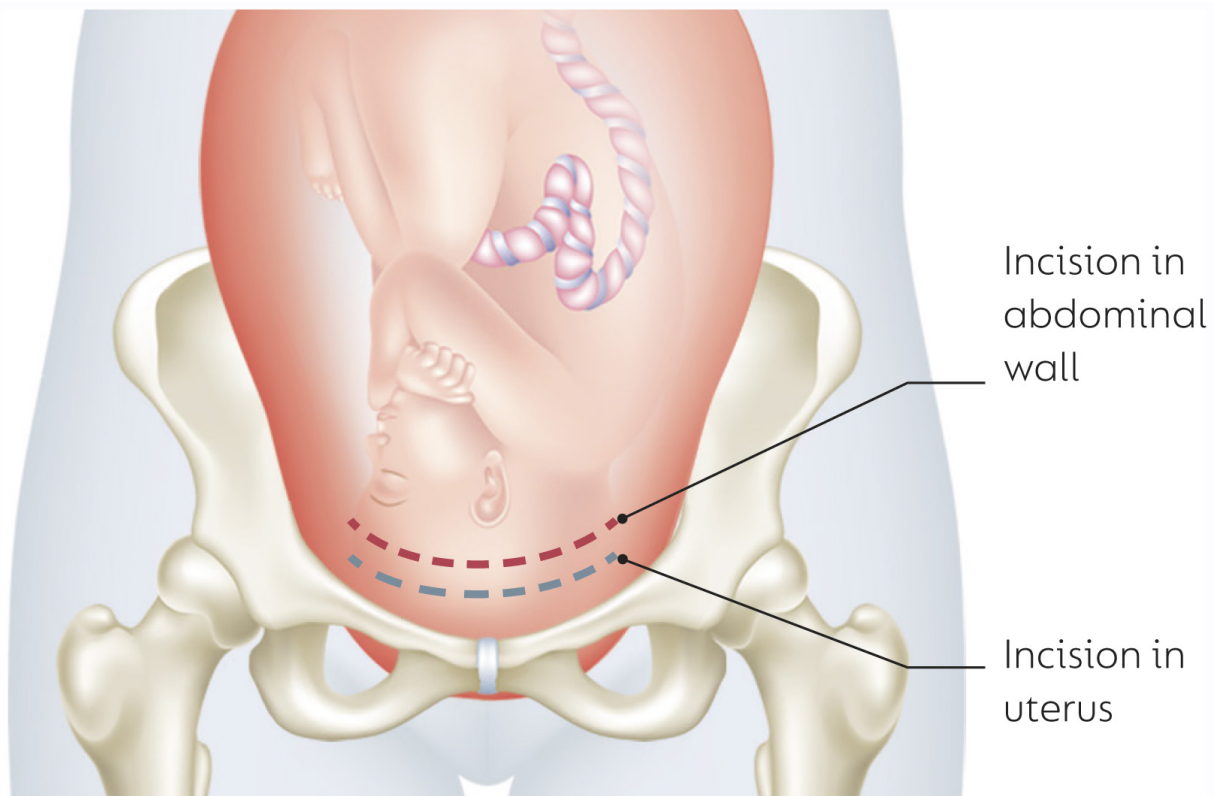


once the uterus has ruptured, the blood supply to the placenta usually stops and the baby becomes short of oxygen. To prevent this from causing a brain injury, the obstetrician must deliver the baby very quickly. For this reason, it is recommended that women who are having a VBAC give birth in a hospital equipped to handle these types of emergencies and have continuous electronic fetal monitoring. The risk of uterine rupture increases twofold if the woman needs to be induced or labor augmented with pitocin.

It can be difficult to choose how to give birth and it is important that you are given the information and the time to decide what is right for you and your baby. Think about what the likelihood is of having an emergency Cesarean. If it is high, you might want to consider having a one that is scheduled. Factors that could affect the likelihood are the reason for your previous Cesarean, whether you have had a previous vaginal birth, how you feel about giving birth, and whether you have had any problems in this pregnancy that may mean it is safer to give birth by Cesarean. On the plus side, if you have a VBAC, your stay in hospital will be shorter, your recovery will be quicker, and if you are planning several babies, you reduce the risk of repeat Cesarean sections.

Your doctor will discuss the risks and benefits of trying to VBAC during your routine prenatal appointments so you have time to consider the best option.



With a low transverse incision, as shown in this artwork, the chance of the Cesarean scar rupturing during a VBAC delivery is very rare. If the scar does open during labor, once that is recognized, an emergency Cesarean is needed, which is why it is important for VBAC deliveries to be closely monitored in an obstetric setting.

Giving your consent

The doctor will need your consent prior to doing a Cesarean. They should explain why the procedure is being recommended and what its benefits and risks are. Ideally, you should have plenty of time to decide whether you want the surgery, although with an